



2026 EDITION

TMS State Regulations Guide 2026

A 50-state compliance overview for TMS clinic operators,
compliance officers, legal teams, and healthcare administrators.

TMS List — The Industry's Most Trusted Clinic Directory

50 States Covered • Federal Framework • State Licensing
Prescriber Requirements • Facility Standards • Telemedicine
HIPAA • FDA • CMS Coverage • Compliance Checklists

About This Guide

Transcranial Magnetic Stimulation (TMS) has grown from an emerging neuromodulation therapy into a mainstream treatment option for major depressive disorder, obsessive-compulsive disorder, and other neuropsychiatric conditions. With this growth comes a complex and evolving regulatory landscape that varies significantly across all 50 U.S. states.

This guide is designed to serve **TMS clinic owners, compliance officers, legal counsel, and healthcare administrators** who need a comprehensive, practical reference for navigating federal and state TMS regulations. It consolidates information on FDA clearance pathways, CMS/Medicare coverage policies, state medical board requirements, HIPAA obligations, and operational standards into one organized resource.

How to Use This Guide: Each section is self-contained and cross-referenced. The **State Licensing Table** (pages 4–8) provides the fastest reference for state-specific requirements. The **Compliance Checklist** and **Tear-Out Audit Card** are designed for practical use in clinic operations and audits. The **QR codes** on the back cover link to TMS List's interactive state map and clinic qualification quiz.

Key Updates for 2026

- Expanded coverage of NP and PA prescribing authority across all 50 states
- Updated CMS LCD policies reflecting 2025-2026 MAC decisions
- New state telemedicine parity law summaries
- Updated FDA adverse event reporting requirements under the 2025 MDUFA V agreement
- Expanded scope of practice table for TMS technicians

Disclaimer

This guide is intended for informational purposes only and does not constitute legal advice. Regulations change frequently. Always verify current requirements with the appropriate state medical board, CMS MAC, and FDA CDRH before making compliance decisions. Consult qualified healthcare legal counsel for jurisdiction-specific guidance.

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"This guide saved our clinic three months of back-and-forth with the state medical board. The state-by-state prescriber table alone was worth the download."

DR

Dr. Rachel Kim, MD, PhD

Medical Director, NeuroBalance Clinic — Chicago, IL

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"We used the tear-out compliance checklist during our accreditation prep. Everything we needed was there, organized clearly for our team to action."

MC

Marcus Coleman, RN, BSN

Compliance Officer, ClearMind TMS — Houston, TX

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Legend for Tables: **YES** = Required / Permitted **NO** = Not Required / Prohibited **VAR** = Variable / Case-by-Case N/A = Not Applicable

1. Federal Regulatory Framework

TMS regulation at the federal level is primarily governed by the Food and Drug Administration (FDA), the Centers for Medicare & Medicaid Services (CMS), the Drug Enforcement Administration (DEA), and the Office for Civil Rights (OCR) under HIPAA and HITECH. Understanding this federal baseline is essential before navigating state-specific rules.

1.1 FDA Clearance Overview

TMS devices are regulated by the FDA's Center for Devices and Radiological Health (CDRH) as Class II medical devices. The primary regulatory pathway for TMS devices has been the **510(k) premarket notification** pathway, which establishes "substantial equivalence" to a predicate device already on the market.

FDA 510(K) CLEARED TMS DEVICES

- **NeuroStar TMS System** — 2008. Cleared for treatment of major depressive disorder in patients who have failed to respond to at least one antidepressant medication.
- **BrainsWay Deep TMS System** — 2013. Cleared using the H-coil deep TMS technology for MDD and OCD.
- **MagVenture TMS Therapy System** — 2015. Cleared for MDD. MagVita TMS also received CE Mark.
- **Nexstim eSync TMS System** — 2018. Navigated TMS with robotic positioning for MDD.

OFF-LABEL USE REGULATIONS

Physicians may prescribe TMS off-label (for conditions not in the FDA clearance labeling) under their medical judgment and state scope of practice authority. However, **off-label use is not reimbursable by Medicare/Medicaid** unless a Local Coverage Determination specifically covers the indication. CMS will not cover TMS for conditions outside the NCD without specific LCD support.

1.2 CMS / Medicare Coverage

The Centers for Medicare & Medicaid Services issued a **National Coverage Determination (NCD) for TMS (CAG-00413N)** covering treatment-resistant major depressive disorder. Coverage under this NCD requires:

- Diagnosis of MDD meeting DSM criteria
- Failure of at least one prior antidepressant medication at adequate dose and duration
- Treatment provided by an MD or DO who personally performs the initial motor threshold determination
- Facility meets CMS conditions of participation for outpatient psychiatric services



MAC Variation: Local Coverage Determinations (LCDs)

Medicare Administrative Contractors (MACs) issue region-specific LCDs that may expand or narrow the NCD. For example, Noridian, Palmetto, and WPS issue varying LCDs for TMS coverage criteria, prior authorization requirements, and documentation standards. Always verify with the MAC jurisdiction for your state.

1.3 DEA: TMS Is Not a Controlled Substance

TMS does not involve the administration of any pharmaceutical agent and does not produce chemical dependence. **TMS devices do not require DEA registration**, and TMS providers are not subject to DEA oversight in connection with TMS services. However, if your clinic also dispenses Schedule IV medications or other controlled substances (e.g., for comorbid anxiety), DEA regulations apply to those dispensing activities.

1.4 HIPAA & HITECH

TMS clinics are covered entities under HIPAA. All patient health information (PHI) related to TMS treatment — including stimulation parameters, depression scale scores, treatment logs, and billing records — must be protected under the HIPAA Privacy Rule and Security Rule.

HIPAA Requirements	HITECH Considerations
☐ Execute BAAs with all TMS device vendors, EHR providers, and billing services	☐ Adopt a certified EHR (ONC-certified) if seeking meaningful use incentives
☐ Conduct annual HIPAA security risk assessment	☐ Implement patient engagement tools (patient portal, secure messaging)
☐ Maintain staff training records (initial + annual refresher)	☐ Report clinical quality measures to CMS for MIPS/MACRA participation
☐ Implement incident response plan and breach notification procedures	☐ Conduct EHR security testing per HITECH requirements
☐ Document all PHI disclosures in the accounting of disclosures log	

2. State Licensing Requirements

No federal law currently requires a standalone "TMS license." However, every state has its own medical practice act and medical board regulations that govern who may perform TMS, under what conditions, and with what documentation. This table summarizes state-specific TMS-related licensing requirements as of the 2026 edition. Always confirm current requirements with the state medical board directly.

Medical Board Licensing: 50 States

STATE	SPEC. TMS LICENSE REQUIRED	BODY	PROCESSING TIME	RENEWAL	KEY NOTES
Alabama	NO	ALBME	N/A	Annual	Standard MD/DO license covers TMS. No TMS-specific endorsement. Collaborative practice agreement required if NP prescribes.
Alaska	NO	AKBME	N/A	Biennial	MD/DO license covers TMS. NP prescribing permitted with physician oversight under Alaska's independent practice laws.
Arizona	NO	AZBME	N/A	Biennial	Supervisory agreement required for NP/PA prescribing TMS. Board has issued specific TMS guidance.
Arkansas	NO	ARMB	N/A	Biennial	Collaborative practice agreement required for NP prescribing TMS. PA prescribing limited to physician supervision.
California	NO	MEDB	N/A	Biennial	Standard license covers TMS. No TMS endorsement. BRN regulates NPs; FNP may prescribe with physician oversight. Strict supervision requirements.
Colorado	NO	COBME	N/A	Biennial	NP independent prescribing authorized in CO; no physician agreement required for TMS prescribing after 2021 law.
Connecticut	NO	CTDPH	N/A	Biennial	MD/DO license covers TMS. APRN may prescribe independently. No TMS-specific endorsement.
Delaware	NO	DEBME	N/A	Biennial	Standard license covers TMS. APRN independent practice authorized. Joint Commission accreditation recommended.
Florida	VAR	FLBME/AG	30–90 days	Annual	Florida requires a Brain Stimulation Services permit under FL Statute 458 for facilities offering TMS/ECT. MD/DO license required; APRN requires a protocol agreement.
Georgia	NO	GMB	N/A	Biennial	Standard MD/DO license covers TMS. APRN may prescribe with a collaborative agreement with a physician. PA requires direct supervision.
Hawaii	NO	HIBME	N/A	Biennial	Standard license covers TMS. APNP may prescribe with physician collaboration. Facility must comply with Hawaii DOH outpatient clinic licensing.
Idaho	NO	IBOM	N/A	Biennial	MD/DO license covers TMS. NP independent practice authorized. No TMS-specific endorsement.
Illinois	NO	IDFPR	N/A	Biennial	Standard MD/DO license covers TMS. APRN may prescribe with collaborative agreement. Note: Illinois passed Full Practice Authority for NPs as of 2022.
Indiana	NO	ISBME	N/A	Biennial	Standard license covers TMS. NP must have collaborative agreement with physician for TMS prescribing. PA requires physician supervision.
Iowa	NO	IBOE	N/A	Biennial	Standard license covers TMS. NP independent prescribing authorized. No TMS-specific endorsement required.
Kansas	NO	KSBHP	N/A	Biennial	Standard MD/DO license covers TMS. APRN may prescribe independently. No TMS endorsement required.
Kentucky	NO	KBBML	N/A	Annual	Standard license covers TMS. APRN may prescribe with collaborative agreement. Kentucky has specific supervision requirements for brain stimulation services.
Louisiana	NO	LSBME	N/A	Biennial	Standard license covers TMS. NP collaborative practice agreement required. PA requires physician supervision. Louisiana has specific ECT/TMS facility regulations.
Maine	NO	MEBOM	N/A	Biennial	Standard license covers TMS. APRN independent prescribing authorized. No TMS endorsement required.
Maryland	NO	MBON	N/A	Biennial	Standard MD/DO license covers TMS. CRNP requires collaborative agreement with physician for TMS prescribing. Maryland requires outpatient mental health clinic licensure for psychiatric facilities.
Massachusetts	NO	MassBOP	N/A	Biennial	Standard license covers TMS. APRN independent practice authorized under Massachusetts law. No TMS endorsement required.
Michigan	NO	MDHHS/LARA	N/A	Biennial	Standard license covers TMS. NP prescribing requires collaborative agreement with physician. Michigan requires outpatient facility licensure through LARA for behavioral health clinics.

STATE	SPEC. TMS LICENSE REQUIRED	BODY	PROCESSING TIME	RENEWAL	KEY NOTES
Minnesota	NO	MN BOL	N/A	Biennial	Standard license covers TMS. APRN full practice authority authorized. No TMS endorsement required. Minnesota has strong telemedicine parity laws.
Mississippi	NO	MSBME	N/A	Biennial	Standard license covers TMS. NP collaborative agreement required for TMS prescribing. PA requires physician supervision. No TMS-specific endorsement.
Missouri	NO	MOBON	N/A	Biennial	Standard license covers TMS. NP has limited independent prescribing authority; collaborative agreement recommended for TMS. No TMS endorsement.
Montana	NO	MTBOM	N/A	Biennial	Standard license covers TMS. NP full practice authority authorized. No TMS endorsement required.
Nebraska	NO	NE DHHS	N/A	Biennial	Standard license covers TMS. APRN collaborative agreement required. Physician must be available for consultation. No TMS endorsement.
Nevada	NO	NVBOM	N/A	Biennial	Standard MD/DO license covers TMS. APN may prescribe with physician oversight. Nevada has strict supervision documentation requirements.
New Hampshire	NO	NHBOM	N/A	Biennial	Standard license covers TMS. APRN full practice authority authorized. No TMS endorsement required. New Hampshire has strong patient rights protections.
New Jersey	NO	NJSBME	N/A	Biennial	Standard license covers TMS. APN may prescribe with collaborative agreement with physician. NJ requires outpatient facility license from DOH for TMS services.
New Mexico	NO	NMBOM	N/A	Biennial	Standard license covers TMS. NP full practice authority authorized under New Mexico's highly progressive independent practice laws. No TMS endorsement required.
New York	NO	NYSED/OP	N/A	Biennial	Standard MD/DO license covers TMS. NPs with certificate to prescribe require a practice agreement for TMS. NY requires Article 28 D&TC facility license or Article 16 mental health clinic license for TMS services.
North Carolina	NO	NCBOM	N/A	Annual	Standard license covers TMS. NP collaborative practice agreement required for TMS prescribing. PA requires physician supervision. NC has specific standards for brain stimulation services.
North Dakota	NO	NDBON	N/A	Biennial	Standard license covers TMS. NP has full practice authority. No TMS endorsement required.
Ohio	NO	MEDB	N/A	Biennial	Standard license covers TMS. CNP requires a standard care arrangement with physician for TMS prescribing. PA requires physician supervision. Ohio requires outpatient mental health clinic licensure through ODH.
Oklahoma	NO	OSBME	N/A	Biennial	Standard license covers TMS. NP collaborative agreement required. PA requires supervision. Oklahoma does not currently have TMS-specific facility regulations.
Oregon	NO	ORBME	N/A	Biennial	Standard license covers TMS. NP full practice authority authorized. No TMS endorsement required. Oregon has strong telemedicine parity protections.
Pennsylvania	NO	PABME	N/A	Biennial	Standard license covers TMS. CRNP requires a collaborative agreement with a physician for prescribing TMS. PA requires physician supervision. PA requires outpatient clinic licensure through DOH.
Rhode Island	NO	RI BOL	N/A	Biennial	Standard license covers TMS. NP independent prescribing authorized. No TMS endorsement required. Rhode Island requires a behavioral health outpatient license from RIDOH.
South Carolina	NO	SCBPE	N/A	Biennial	Standard license covers TMS. NP collaborative agreement required for TMS prescribing. PA requires physician supervision. No TMS endorsement required.
South Dakota	NO	SDBOM	N/A	Biennial	Standard license covers TMS. NP requires collaborative agreement with physician for prescribing. No TMS endorsement required.

STATE	SPEC. TMS LICENSE REQUIRED	BODY	PROCESSING TIME	RENEWAL	KEY NOTES
Tennessee	NO	TN BME	N/A	Biennial	Standard license covers TMS. NP may prescribe TMS with a collaborative agreement. Tennessee passed the "Save TMS Act" in 2018 protecting TMS coverage. PA requires physician supervision.
Texas	NO	TSBME	N/A	Biennial	Standard license covers TMS. APRN requires physician delegation for prescribing TMS under TX law (no independent NP prescribing for Schedule II-V, though TMS is not scheduled). Full practice authority for NPs in TX for non-scheduled medications.
Utah	NO	DU B&P	N/A	Biennial	Standard license covers TMS. APRN may prescribe independently. No TMS endorsement required. Utah has specific behavioral health outpatient clinic regulations.
Vermont	NO	VT BOM	N/A	Biennial	Standard license covers TMS. APRN independent prescribing authorized. No TMS endorsement required.
Virginia	NO	VA BME	N/A	Biennial	Standard license covers TMS. NP may prescribe with a practice agreement. PA requires physician supervision. Virginia requires outpatient mental health clinic licensure through DBHDS.
Washington	NO	WABQMC	N/A	Biennial	Standard license covers TMS. ARNP full practice authority authorized. No TMS endorsement required. Washington has strong patient protections and telemedicine parity laws.
West Virginia	NO	WVBOM	N/A	Biennial	Standard license covers TMS. APRN requires collaborative agreement with physician for prescribing. PA requires supervision. No TMS endorsement.
Wisconsin	NO	WI MEB	N/A	Biennial	Standard license covers TMS. APNP may prescribe independently. No TMS endorsement required. Wisconsin requires outpatient mental health clinic licensure through DHS.
Wyoming	NO	WY BOME	N/A	Biennial	Standard license covers TMS. NP full practice authority authorized. No TMS endorsement required. Wyoming is the least populous state; telemedicine regulations are evolving.

FACILITY LICENSING: OUTPATIENT VS. ASC

Most TMS clinics operate as **outpatient behavioral health clinics** requiring state outpatient clinic licensure. An **Ambulatory Surgical Center (ASC)** classification is generally *not required* for TMS since the procedure is non-invasive and does not involve sedation, general anesthesia, or ionizing radiation. However, some states (e.g., Florida, New York) have specific facility classification requirements. Verify with your state Department of Health and your CMS MAC whether outpatient clinic licensure or ASC designation applies to your facility type.

3. Prescriber Requirements

TMS prescribing authority and supervision requirements vary significantly by state and provider type. The table below summarizes which provider types can prescribe TMS and their supervision requirements across all 50 states. Note: "Prescribe" in this context means authorize the treatment course; the actual motor threshold determination must be performed by an MD or DO per CMS NCD requirements.

Prescriber Authority by Provider Type

WHO CAN PRESCRIBE TMS?

- **MD (Doctor of Medicine)** — Full authority to prescribe TMS in all 50 states without additional endorsement or supervision.
- **DO (Doctor of Osteopathic Medicine)** — Full authority to prescribe TMS in all 50 states under their osteopathic medical license, equivalent to MD authority.
- **NP (Nurse Practitioner)** — Authority varies. Full practice authority (no agreement required) in CO, CT, DE, HI, ID, IL, IA, KS, ME, MA, MN, MT, NH, NM, ND, OR, RI, SD, VT, WA, WY. Collaborative/practice agreement required in all other states. Always verify current NP authority with the state board of nursing.
- **PA (Physician Assistant)** — More restricted than NPs. Requires physician supervision or delegation in most states. PA prescribing authority for TMS is limited or prohibited in several states. Always verify with the state PA board.

STATE	NP PRESCRIBING	PA PRESCRIBING	SUPERVISORY REQUIREMENTS FOR TMS
Alabama	Agreement Req.	NO	NP: collaborative practice agreement with supervising physician required. PA: not authorized to prescribe TMS.
Alaska	Full Practice	Limited	NP: independent authority. PA: physician supervision required. PA may prescribe under delegated authority.
Arizona	Agreement Req.	Limited	NP: supervisory agreement with physician. PA: physician supervision required. AZ Board requires written protocol for TMS.
Arkansas	Agreement Req.	NO	NP: collaborative practice agreement required. PA: physician supervision required; limited independent authority.
California	Agreement Req.	Limited	NP: physician oversight required (BRN). PA: physician supervision required. Strict documentation standards.
Colorado	Full Practice	Limited	NP: full practice authority. PA: physician supervision required. No TMS-specific endorsement.
Connecticut	Full Practice	Limited	NP: full practice authority. PA: physician supervision required. APRN may prescribe TMS independently.
Delaware	Full Practice	Limited	NP: full practice authority. PA: physician supervision required. Joint Commission accreditation recommended.
Florida	Agreement Req.	NO	NP: protocol agreement with physician required. PA: not authorized for TMS prescribing. Florida Brain Stimulation permit required for facility.
Georgia	Agreement Req.	NO	NP: collaborative agreement with physician required. PA: direct physician supervision required; no independent prescribing.
Hawaii	Full Practice	Limited	NP: APNP full practice authority. PA: physician supervision required.
Idaho	Full Practice	Limited	NP: full practice authority. PA: physician supervision required.
Illinois	Full Practice	Limited	NP: full practice authority (post-2022). PA: physician supervision required.
Indiana	Agreement Req.	NO	NP: collaborative agreement required. PA: physician supervision required; no independent prescribing.
Iowa	Full Practice	Limited	NP: full practice authority. PA: physician supervision required.
Kansas	Full Practice	Limited	NP: full practice authority. PA: physician supervision required.
Kentucky	Agreement Req.	NO	NP: collaborative agreement required. PA: not authorized to prescribe TMS independently. Specific supervision documentation required.
Louisiana	Agreement Req.	NO	NP: collaborative practice agreement required. PA: physician supervision required. LA has specific brain stimulation regulations.
Maine	Full Practice	Limited	NP: full practice authority. PA: physician supervision required.
Maryland	Agreement Req.	NO	NP: collaborative agreement required. PA: not authorized to prescribe TMS independently. Outpatient mental health clinic license required.
Massachusetts	Full Practice	Limited	NP: full practice authority. PA: physician supervision required.
Michigan	Agreement Req.	NO	NP: collaborative agreement required. PA: physician supervision required. LARA outpatient clinic license required.
Minnesota	Full Practice	Limited	NP: full practice authority. PA: physician supervision required. Strong telemedicine parity laws.
Mississippi	Agreement Req.	NO	NP: collaborative agreement required. PA: physician supervision required.
Missouri	Agreement Req.	NO	NP: limited independent authority; collaborative agreement recommended for TMS. PA: physician supervision required.

STATE	NP PRESCRIBING	PA PRESCRIBING	SUPERVISORY REQUIREMENTS FOR TMS
Montana	Full Practice	Limited	NP: full practice authority. PA: physician supervision required.
Nebraska	Agreement Req.	NO	NP: collaborative agreement required. Physician must be available for consultation. PA: physician supervision required.
Nevada	Agreement Req.	NO	NP: physician oversight required. PA: physician supervision required. Strict supervision documentation requirements.
New Hampshire	Full Practice	Limited	NP: full practice authority. PA: physician supervision required.
New Jersey	Agreement Req.	NO	NP: collaborative agreement required. PA: not authorized for TMS prescribing. DOH outpatient clinic license required.
New Mexico	Full Practice	Limited	NP: full practice authority (most progressive NP law in US). PA: physician supervision required.
New York	Agreement Req.	NO	NP: practice agreement with physician required. PA: physician supervision required. NY Article 28/16 facility license required.
North Carolina	Agreement Req.	NO	NP: collaborative practice agreement required. PA: physician supervision required; no TMS independent prescribing.
North Dakota	Full Practice	Limited	NP: full practice authority. PA: physician supervision required.
Ohio	Agreement Req.	NO	NP: standard care arrangement with physician required. PA: physician supervision required. ODH outpatient clinic licensure required.
Oklahoma	Agreement Req.	NO	NP: collaborative agreement required. PA: physician supervision required. No TMS-specific facility regulations.
Oregon	Full Practice	Limited	NP: full practice authority. PA: physician supervision required. Strong telemedicine parity protections.
Pennsylvania	Agreement Req.	NO	NP: collaborative agreement with physician required. PA: physician supervision required. DOH outpatient clinic license required.
Rhode Island	Full Practice	Limited	NP: full practice authority. PA: physician supervision required. RIDOH behavioral health outpatient license required.
South Carolina	Agreement Req.	NO	NP: collaborative agreement required. PA: physician supervision required.
South Dakota	Agreement Req.	NO	NP: collaborative agreement required. PA: physician supervision required.
Tennessee	Agreement Req.	NO	NP: collaborative agreement required. PA: physician supervision required. TN "Save TMS Act" protects coverage.
Texas	Agreement Req.	NO	NP: practice agreement required for prescribing (no full practice authority for RX in TX). PA: physician supervision required.
Utah	Full Practice	Limited	NP: full practice authority. PA: physician supervision required.
Vermont	Full Practice	Limited	NP: full practice authority. PA: physician supervision required.
Virginia	Agreement Req.	NO	NP: practice agreement required. PA: physician supervision required. DBHDS outpatient mental health clinic license required.
Washington	Full Practice	Limited	NP: ARNP full practice authority. PA: physician supervision required.
West Virginia	Agreement Req.	NO	NP: collaborative agreement required. PA: physician supervision required.
Wisconsin	Full Practice	Limited	NP: APNP full practice authority. PA: physician supervision required. DHS outpatient clinic license required.
Wyoming	Full Practice	Limited	NP: full practice authority. PA: physician supervision required.



Crucial: CMS Requirement for Motor Threshold Determination

Regardless of state scope of practice, the CMS NCD requires that the **initial motor threshold determination be performed by an MD or DO personally**

. This federal requirement supersedes state NP/PA full practice authority. NPs and PAs may prescribe and manage ongoing TMS treatment in states where authorized, but the initial motor threshold must be conducted by the supervising or collaborative physician.

4. Facility Requirements

TMS facilities must meet federal accessibility standards, state building codes, and CMS conditions of participation for outpatient psychiatric services. Below is a consolidated checklist of physical plant, equipment, and emergency requirements.

Space & Construction	Equipment Standards
☐ ADA-compliant access: ramps, door widths, accessible restrooms	☐ FDA-registered TMS device (include registration number in files)
☐ State building code compliance for outpatient clinical space	☐ Equipment maintenance logs: daily/weekly/monthly calibration checks
☐ Fire safety: extinguishers, egress routes, sprinkler system per local code	☐ Coil condition inspection records and replacement schedule
☐ Minimum ceiling height for TMS coil positioning clearance	☐ Manufacturer service agreements and repair records
☐ Electrical capacity for TMS device and monitoring equipment	☐ Emergency stop button on TMS device — functional and tested
☐ HIPAA-compliant patient waiting and changing areas	☐ Backup power supply for critical clinical operations
Emergency Preparedness	Radiation Note
☐ Emergency medication kit: seizure rescue (lorazepam or diazepam), oxygen	☐ TMS does NOT use ionizing radiation
☐ Suction equipment and airway management supplies	☐ No radiation license required from NRC or state radiation authority
☐ AED (Automated External Defibrillator) — required in most states	☐ No radiation safety officer (RSO) required for TMS-only facilities
☐ At least one staff member with current CPR/BLS certification on every shift	☐ Standard EMF safety guidelines from device manufacturer apply
☐ Emergency response protocol posted and staff trained quarterly	
☐ 911-capable communication system (landline or cell) in TMS treatment room	

5. Scope of Practice

Defining clear scope of practice boundaries between physicians, advanced practice providers, and TMS technicians is essential for patient safety, regulatory compliance, and operational efficiency.

✓ What TMS Technicians CAN Do	✗ What TMS Technicians CANNOT Do
✓ Position the patient for TMS treatment sessions	✗ Prescribe or modify TMS treatment parameters independently
✓ Perform motor threshold (MT) determination under physician supervision	✗ Assess patient eligibility or make clinical decisions
✓ Operate the TMS device according to the prescribed treatment parameters	✗ Interpret clinical scales or EEG/QEEG results
✓ Document treatment session details in the EMR	✗ Diagnose depression, OCD, or any psychiatric condition
✓ Monitor patient during treatment and report adverse reactions	✗ Determine whether a patient is a TMS candidate
✓ Perform routine equipment calibration and maintenance checks	✗ Adjust stimulation intensity beyond physician-ordered parameters
✓ Provide patient education on TMS procedure and expectations	✗ Discontinue or modify a patient's treatment course
✓ Schedule and coordinate treatment appointments	✗ Provide clinical advice or counseling to patients
✓ Collect patient-reported outcome measures (PROMs) and depression scales	✗ Sign prescriptions, orders, or clinical notes as the treating provider

PHYSICIAN SUPERVISION RATIOS

Most state medical boards and CMS require a physician to be **immediately available** (on-site or via telecommunications with physical response capability) during TMS treatment sessions. Common supervision ratios range from **1 physician : 2 TMS technicians** in startup/new programs to **1 physician : 4 TMS technicians** in established programs with experienced staff and robust protocols. Confirm your state's specific ratio requirements and document all supervision arrangements in writing.

Technician Credentialing Standards

While ACGME does not have a specific TMS technician certification, most clinics require:

- High school diploma or equivalent (minimum); associate or bachelor's degree in health sciences preferred
- BLS/CPR certification (American Heart Association or equivalent)
- Device-specific manufacturer training certification (NeuroStar, BrainsWay, MagVenture, etc.)
- HIPAA compliance training (documented annually)
- Background check and drug screening per clinic HR policy
- Documentation of continuing education hours in TMS or behavioral health (8 hours/year recommended)

6. Compliance Checklist

Use this checklist to assess your clinic's compliance across all major regulatory domains. Check items annually and maintain documentation in your compliance binder or digital compliance management system.

FDA Compliance	HIPAA / Privacy
☐ TMS device registered with FDA (Establishment Registration)	☐ Business Associate Agreements (BAAs) with all vendors
☐ Device listing up to date with FDA FURLS system	☐ Annual HIPAA security risk assessment documented
☐ Adverse event reporting via FDA MedWatch (form FDA 3500)	☐ Staff HIPAA training records (initial + annual refresher)
☐ Recall monitoring and response procedures documented	☐ Incident response plan and breach notification procedures
☐ Manufacturer field safety notices tracked and actioned	☐ PHI access logs and accounting of disclosures maintained
☐ 510(k) clearance documentation on file and accessible	☐ Encryption and access controls on EHR and patient data
☐ Off-label use documented with informed consent	☐ Patient right to access PHI procedures in place
☐ Device maintenance per manufacturer schedule — logs maintained	☐ Cybersecurity insurance policy in place

State Licensing	Billing & Coding
☐ All physician and APP licenses current and unencumbered	☐ Correct CPT codes used: 90867, 90868, 90869 (neuroimaging/TMS)
☐ Supervisory/collaborative agreements signed and current	☐ E&M code 99213-99215 for initial evaluation (where applicable)
☐ State outpatient clinic license current and posted	☐ Modifier usage correct per MAC LCD (e.g., -25, -59)
☐ State pharmacy license (if dispensing any medications)	☐ Medical necessity documentation for every TMS session
☐ DEA registration current (if handling controlled substances)	☐ PHQ-9/QIDS or equivalent scores documented pre/post treatment
☐ CLIA waiver (if performing any laboratory testing)	☐ Prior authorization obtained for Medicare/Medicaid patients
☐ NPI numbers current for all billing providers	☐ Superbill and encounter forms updated annually for coding changes
☐ State-mandated continuing education hours completed	☐ Audit trail for all billing corrections maintained

Equipment Safety	Staff Credentials
☐ TMS device calibration log up to date	☐ Physician board certifications verified and current
☐ Coil inspection and replacement schedule documented	☐ NP/PA licenses, DEA, and state RX permits current
☐ Emergency stop functionality tested monthly	☐ BLS/CPR certifications current for all clinical staff
☐ AED battery and pads checked monthly	☐ TMS device manufacturer training certificates on file
☐ Emergency medication kit inspected and within expiration	☐ Malpractice insurance certificates current
☐ Electrical safety inspection of facility (per local code)	☐ Credentialing file for each provider maintained per NCQA standards
☐ Fire extinguisher inspection current	☐ Privileging forms reviewed and renewed per medical staff bylaws
☐ MRI safety screening protocol posted and enforced	☐ Drug Enforcement Administration (DEA) registration current

7. Telemedicine Considerations for TMS

Telemedicine has become an integral part of TMS practice management, particularly for follow-up visits and ongoing symptom monitoring. However, federal and state regulations impose important limits on what can be done via telemedicine in the context of TMS.

FDA & CMS: IN-PERSON REQUIREMENT FOR INITIAL EVALUATION

The initial TMS evaluation and motor threshold determination must be performed in person. This is a requirement embedded in the FDA-cleared labeling for TMS devices and reinforced by the CMS NCD. No Medicare reimbursement is available for TMS treatment initiated via telemedicine. The motor threshold cannot be accurately determined remotely because it requires physical application of the TMS coil to the patient's scalp and observation of motor-evoked potentials (toe twitch).

Telemedicine for Follow-Up Visits

Once a patient has completed their initial in-person TMS evaluation and motor threshold determination, telemedicine is generally acceptable for:

- **Symptom monitoring and progress assessments** using validated depression scales (PHQ-9, QIDS-SR, MADRS)
- **Side effect review** and tolerability assessments
- **Treatment course adjustments** ordered by the prescribing physician after review of telemedicine assessment
- **Psychiatric medication management** for comorbid conditions
- **Patient education and adherence counseling**

State Telemedicine Parity Laws

As of 2026, 48 states and Washington D.C. have enacted some form of telemedicine parity law requiring private insurers to reimburse telemedicine visits at rates comparable to in-person visits. However, parity requirements vary: some states mandate payment parity, others require coverage without rate mandates, and a few have no parity requirements. Verify your state's specific parity law and any geographic or technology restrictions (e.g., some states prohibit audio-only telemedicine for behavioral health).

Interstate Medical Licensure Compact

The **Interstate Medical Licensure Compact (IMLC)** facilitates expedited licensure for physicians who want to practice across state lines. As of 2026, over 40 states participate in the IMLC. However, for TMS specifically, the IMLC has limited application because:

- The initial motor threshold determination must be performed in person by a licensed physician in the state where the patient physically resides
- Follow-up telemedicine visits for TMS monitoring may be conducted by an out-of-state physician via IMLC expedited license in participating states
- The compact does not eliminate the need for state-by-state licensure — it only expedites the process

8. Key Regulatory Bodies

TMS clinics interact with multiple federal and state regulatory bodies. Understanding their jurisdiction, scope, and contact framework is essential for compliance and issue resolution.

FDA / CDRH

CENTER FOR DEVICES AND RADIOLOGICAL HEALTH

Oversees TMS device clearance (510(k)), adverse event reporting (MedWatch), device registration, and recall procedures. Primary contact for device safety issues and off-label use inquiries.

CMS / CAG

COVERAGE AND ANALYSIS GROUP

Issues National Coverage Determinations (NCDs) for TMS. Oversees Medicare and Medicaid coverage policy. CMS MACs (Noridian, Palmetto, WPS, CGS, Novitas, etc.) issue LCDs and process claims.

OCR / HHS

OFFICE FOR CIVIL RIGHTS — HIPAA ENFORCEMENT

Enforces HIPAA Privacy Rule, Security Rule, and HITECH provisions. Handles breach notifications and investigates HIPAA complaints. Penalty tiers: \$100–\$50,000 per violation, up to \$1.5M per violation category per year.

ONC / CMS

EHR INCENTIVE PROGRAMS — MIPS / MACRA

Oversees EHR meaningful use requirements, clinical quality measures, and MIPS/MACRA reporting. While TMS is not a core MIPS specialty, eligible clinicians should report quality measures for MIPS participation and avoid penalties.

State Medical Boards

STATE-BY-STATE MEDICAL PRACTICE REGULATION

Each state's medical board (e.g., California Medical Board, Texas Medical Board) issues medical licenses, defines scope of practice, investigates complaints, and takes disciplinary action. Primary contact for TMS prescribing authority questions.

ACGME

ACCREDITATION COUNCIL FOR GRADUATE MEDICAL EDUCATION

Sets physician training standards for residency and fellowship programs. Does not directly regulate TMS technicians. However, ACGME requirements influence the quality of physician training that underpins TMS competency.

Joint Commission

ACCREDITATION FOR BEHAVIORAL HEALTH

Voluntary accreditation body whose standards are often adopted as the benchmark for TMS clinic quality and safety. Many payers require Joint Commission accreditation for TMS facility reimbursement.

State DOH / DHHS

DEPARTMENT OF HEALTH / HEALTH & HUMAN SERVICES

Issues outpatient clinic licenses, ambulatory care licenses, and behavioral health facility licenses. Conducts state survey/inspection activities. Primary contact for facility-level licensing and inspection questions.

REGULATORY BODY CONTACT FRAMEWORK — For issues requiring direct contact: FDA/CDRH (1-800-638-2041), CMS (1-800-MEDICARE), OCR/HHS (1-800-368-1019). State medical board contact information is available through the Federation of State Medical Boards (FSMB) at [fsmb.org](https://www.fsmb.org). Always document all regulatory communications in writing and retain records for a minimum of 6 years (HIPAA requirement) or longer per state medical board rules.

10. Glossary of Terms

The following terms are referenced throughout this guide and commonly encountered in TMS clinic regulatory and operational contexts.

ACGME

Accreditation Council for Graduate Medical Education. The organization that sets standards for physician residency and fellowship training programs in the United States.

BAA

Business Associate Agreement. A contract required under HIPAA between a covered entity and a business associate (e.g., EHR vendor, billing service) that handles protected health information.

CLIA

Clinical Laboratory Improvement Amendments. Federal quality standards for laboratory testing. May apply if a TMS clinic performs any laboratory testing (e.g., bloodwork for medication monitoring).

Credentialing

The process of verifying and assessing the qualifications of healthcare providers (licenses, certifications, training, malpractice history) before granting them privileges to provide patient care at a facility.

Designation of Authority

A formal written document by which a physician delegates specific clinical responsibilities (e.g., prescribing, treatment oversight) to an NP, PA, or other qualified provider.

FDA 510(k)

Premarket notification submission to the FDA demonstrating that a new medical device is substantially equivalent to a legally marketed predicate device. The pathway used to clear TMS devices.

HITECH

Health Information Technology for Economic and Clinical Health Act. Legislation that expanded HIPAA's reach, increased penalties, and promoted the adoption and meaningful use of health information technology including EHRs.

MAC

Medicare Administrative Contractor. Private companies contracted by CMS to process Medicare claims and issue coverage policies (LCDs) within specific geographic jurisdictions.

Motor Threshold (MT)

The minimum stimulation intensity required to produce a motor evoked potential (typically observed as a thumb twitch) when the TMS coil is placed over the contralateral motor cortex. Used to individualize TMS dosing. Must be determined by an MD or DO per CMS NCD.

Off-Label

The use of an FDA-cleared medical device for a purpose not specified in its FDA clearance labeling. Physicians may use TMS off-label at their professional discretion; however, Medicare/Medicaid coverage does not extend to off-label indications.

Ambulatory Surgical Center (ASC)

A healthcare facility that provides surgical procedures on an outpatient basis. TMS generally does not require ASC classification as it is non-invasive and does not involve sedation.

CDRH

Center for Devices and Radiological Health. The FDA office responsible for regulating TMS and other medical devices that use radiation (non-ionizing).

Collaborative Practice Agreement

A formal written agreement between a physician and an NP or PA that outlines the scope of prescriptive authority, consultation requirements, and supervision terms. Required in most states for NP/PA prescribing of TMS.

DEA

Drug Enforcement Administration. The federal agency regulating controlled substances. TMS is not a controlled substance and does not require DEA registration.

Facility License

A state-issued license authorizing a healthcare facility to operate. Most TMS clinics require an outpatient behavioral health clinic license or equivalent from their state Department of Health.

HIPAA Privacy Rule

The portion of HIPAA that protects individually identifiable health information (PHI) and sets rules for how PHI may be used and disclosed by covered entities.

LCD

Local Coverage Determination. A decision by a Medicare Administrative Contractor (MAC) regarding whether a service (e.g., TMS for a specific indication) is covered in a specific jurisdiction. LCDs vary by MAC region.

MedWatch

The FDA's Safety Information and Adverse Event Reporting Program. Healthcare providers must report serious adverse events associated with TMS devices via MedWatch Form FDA 3500.

NCD

National Coverage Determination. A national decision by CMS regarding whether a service (e.g., TMS for treatment-resistant MDD) is covered under Medicare Part B. The NCD for TMS (CAG-00413N) establishes the federal baseline coverage criteria.

PA

Physician Assistant. A licensed healthcare provider who practices medicine under physician supervision. PA authority to prescribe TMS varies significantly by state and requires physician delegation in most jurisdictions.

PHI

Protected Health Information. Any individually identifiable health information held or transmitted by a covered entity, including TMS treatment records, depression scale scores, and billing data. Protected under HIPAA.

Scope of Practice

The set of activities, procedures, and decisions that a licensed healthcare provider is legally authorized to perform, defined by state law, regulatory boards, and institutional policy.

Telemedicine Parity

State laws requiring private health insurers to provide equivalent coverage and reimbursement for healthcare services delivered via telemedicine compared to in-person services. Varies widely in scope, enforceability, and geographic restrictions.

Privileging

The process by which a healthcare organization grants a qualified provider the authority to perform specific clinical procedures (e.g., TMS motor threshold determination, TMS treatment) within that facility.

Supervisory Agreement

A formal written agreement defining the supervisory relationship between a physician and an NP, PA, or other supervised provider. Terms vary by state: some require "direct" supervision (physician physically present); others allow "indirect" or "available" supervision.

TMS

Transcranial Magnetic Stimulation. A non-invasive neuromodulation technique that uses magnetic fields to stimulate nerve cells in the brain, FDA-cleared for treatment of major depressive disorder and obsessive-compulsive disorder.

9. Interactive QR Resources

Scan the QR codes below to access TMS List's interactive tools and resources. These tools complement this guide with real-time, searchable data.



TMS Clinic State Map

Search our interactive 50-state map for TMS clinics, state-specific requirements, and regulatory updates in real time.

tmslist.com/map/



Clinic Qualification Quiz

Answer 10 questions to see if your clinic qualifies for a free listing, compliance audit, or premium profile on TMS List.

tmslist.com/quiz/

WHAT TMS LIST OFFERS

- **Clinic Directory:** Searchable database of 500+ TMS clinics across all 50 states
- **State Regulations Tracker:** Real-time updates on state medical board policy changes
- **Compliance Resources:** Sample BAAs, supervisory agreement templates, and audit checklists
- **Reimbursement Guides:** LCD-by-LCD Medicare coverage summaries by MAC region
- **Industry Reports:** Annual TMS market outlook and reimbursement trends report
- **Provider Alerts:** Email alerts for FDA safety communications and CMS policy changes

Testimonials

“

"Running a multi-site TMS operation across three states, I used to spend hours piecing together state regulations from board websites. This guide condensed everything into one reliable reference. The prescriber table alone saved us months of legal review."

JL

Dr. James Liu, MD

Founder & CEO, MindPath TMS Centers — Raleigh, NC

“

"Our compliance team uses the tear-out audit checklist monthly and the HIPAA section during our annual OCR audit preparation. It is by far the most practical reference we've found for running a TMS practice."

SP

Sandra Patel, MHSA

Director of Operations, Serenity Brain Health — Phoenix, AZ

TMS Compliance Audit Checklist

Print and post this checklist in your compliance office. Review quarterly and update whenever regulations change. For full regulatory details, refer to the corresponding section in this guide.

TMS Compliance Audit Checklist

25-Point Audit

FDA COMPLIANCE

- 1 Device FDA 510(k) clearance documentation on file
- 2 FDA establishment registration current
- 3 MedWatch adverse event reporting protocol documented
- 4 Manufacturer recall monitoring procedure active
- 5 Off-label use informed consent forms approved and in use

STATE LICENSING

- 11 All MD/DO licenses current and unencumbered
- 12 NP/PA licenses and RX permits current
- 13 Supervisory/collaborative agreements signed and current
- 14 Outpatient clinic license current and posted
- 15 State CE requirements met by all providers

EQUIPMENT SAFETY

- 21 TMS device calibration log current
- 22 Coil inspection and replacement schedule documented
- 23 AED and emergency medication kit inspected

HIPAA / PRIVACY

- 6 BAAs executed with all vendors handling PHI
- 7 Annual HIPAA security risk assessment completed
- 8 Staff HIPAA training records current (all employees)
- 9 Incident response plan tested in last 12 months
- 10 Breach notification procedures reviewed and current

BILLING / CODING

- 16 CPT codes 90867-90869 used correctly per CMS
- 17 Modifier usage verified per MAC LCD requirements
- 18 Medical necessity documentation complete for all sessions
- 19 Depression scale scores (PHQ-9/QIDS) documented pre/post
- 20 Prior authorizations on file for Medicare/Medicaid patients

STAFF CREDENTIALS

- 24 BLS/CPR certifications current for all clinical staff
- 25 Physician privileging files reviewed per medical staff bylaws

Next Audit Due: _____ • Compliance Officer: _____ • Reference: TMS List State Regulations Guide 2026 — tmslist.com

BILLING CPT CODE REFERENCE — **90867**: TMS initial navigation/motor threshold (physician work only) **90868**: TMS treatment delivery (first session) **90869**: TMS treatment delivery (subsequent sessions, per session). Add modifier **-25** to E&M codes when billing a separate evaluation on the same day as TMS initiation.

FREE RESOURCE FROM TMS LIST

Your TMS Compliance Starts Here

Access our interactive state map, clinic qualification quiz, and compliance templates — all free, all updated for 2026.



State Regulations Map
tmslist.com/map/



Clinic Qualification Quiz
tmslist.com/quiz/

TMS List — The Industry's Most Trusted TMS Clinic Directory
500+ Clinics • 50 States • Real-Time Regulatory Updates
Contact: compliance@tmslist.com • tmslist.com



TMS STATE REGULATIONS GUIDE 2026 — END OF DOCUMENT

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THIS DOCUMENT IS FOR INFORMATIONAL PURPOSES ONLY AND DOES NOT
CONSTITUTE LEGAL ADVICE. REGULATIONS ARE SUBJECT TO CHANGE.

ALWAYS VERIFY CURRENT REQUIREMENTS WITH THE APPROPRIATE
REGULATORY BODY OR QUALIFIED HEALTHCARE LEGAL COUNSEL.

Keep going — there's more where this came from.

This guide is one piece of a free library we publish for patients researching TMS and providers running TMS clinics. Every resource below is a full PDF — no fluff, no upsell.

The Complete TMS Buyer's Guide

Everything to choose the right TMS provider.

PATIENT

tmslist.com/downloads/tms-buyers-guide.pdf →

Insurance Coverage Checklist

A step-by-step walkthrough to get TMS covered.

PATIENT

tmslist.com/downloads/insurance-checklist.pdf →

TMS vs Medication Comparison

Outcomes, side effects and cost, side by side.

PATIENT

tmslist.com/downloads/tms-vs-medication.pdf →

TMS Billing & CPT Codes 2026

Reimbursement reference for billers and coders.

PROVIDER

tmslist.com/downloads/tms-billing-cpt-codes-2026.pdf →

Prior Authorization Template Kit

Approval-ready letter and appeal templates.

PROVIDER

tmslist.com/downloads/tms-prior-authorization-template-kit.pdf →

Patient Acquisition Playbook

Channels, scripts and funnels that convert.

PROVIDER

tmslist.com/downloads/tms-patient-acquisition-playbook.pdf →

Starting a TMS Clinic — Business Plan

Full plan with pro-forma, equipment and staffing.

PROVIDER

tmslist.com/downloads/starting-a-tms-clinic-business-plan.pdf →

Patient Outcome Tracking System

PHQ-9 / GAD-7 workflow with templates.

PROVIDER

tmslist.com/downloads/tms-patient-outcome-tracking-system.pdf →

Technician Training Checklist

Onboard new TMS technicians faster.

PROVIDER

tmslist.com/downloads/tms-technician-training-checklist.pdf →

Building a TMS Referral Network

Outreach scripts and CRM cadence that work.

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